

TOOLS, NOT THEORY

A working manual for the gap between understanding a pattern and being able to move inside it

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Tools, Not Theory: A Working Manual

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Health notice. This book offers general psychoeducation (structured information about how a pattern works and what helps) and self-help strategies drawn from behavioural activation, cognitive behavioural therapy (CBT), acceptance and commitment therapy (ACT) and compassion-focused therapy (CFT). It does not diagnose or treat any condition and is not a substitute for care from a qualified health professional. If your symptoms are severe, persistent or worsening, or if you have thoughts of harming yourself, seek professional help. The author and publisher accept no liability for actions taken on the basis of this book.

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Composite examples. Apart from the first worked example in Chapter 03, which is the author's own, all examples are composites and do not describe real individuals.

BEFORE ANYTHING

Read This First: If You're Not Safe Right Now

Crisis numbers checked 27 September 2026.

If you are thinking about ending your life, if you have already harmed yourself or taken something, or you're not sure you can keep yourself safe, stop reading. This book is the wrong tool for this moment. A person is the right one. That includes hurting yourself without wanting to die: this page is for that too.

- If you might act on these thoughts soon, or you've already hurt yourself, call your local emergency number:
 - US and Canada: 911
 - UK: 999
 - EU: 112
 - Australia: 000
 - UAE: 999 for police, 998 for an ambulance. If you need medical help rather than the police, call 998.
- To talk to someone now:
 - US: call or text 988, the 988 Suicide & Crisis Lifeline. 24 hours.
 - Canada: call or text 988, the 9-8-8 Suicide Crisis Helpline. 24 hours.
 - UK and Ireland: call Samaritans on 116 123. Free, 24 hours.
 - Australia: call Lifeline on 13 11 14, or text 0477 13 11 14.
 - Anywhere else, including the UAE: findahelpline.com lists free helplines by country.
- Tell one person, today, in plain words. "I'm not safe tonight and I need you with me" is enough. It doesn't need to be explained well. If the people closest to you aren't safe to tell, because of how they might react or what it might cost you, choose someone else: a friend, a doctor, a colleague, or someone outside your household you trust.
- Make the space around you safer by putting distance between yourself and anything you might use to hurt yourself, even if only for tonight.

Thoughts like these are a symptom. They are common in depression and they are treatable. They are also a reason to get help today, not after you've finished a chapter.

On a steadier day, fill in the Safety Plan in Appendix C4. Having it written before you need it is one of the most protective things in this book.

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ORIENTATION

Introduction: How to Use This

This is a manual you run, not a book you finish. If you read it end to end and change nothing, that's not unusual. It's just not what the book is for. Come back to any chapter and try one tool; that's the point where it starts working.

It is built on one distinction. Knowing why a pattern exists is mostly declarative knowledge — facts and explanations — and it updates when something is explained well. Being able to move differently inside that pattern behaves more like a skill, and it updates mainly through repetition. You may well have the first already. Every tool here targets the second.

Who This Is For

Maybe you've read the books. You can name your patterns, sometimes in their clinical vocabulary, and none of it has changed what happens on an ordinary morning when the thing is in front of you and you don't begin. Or maybe you've never read one, and someone handed you this. Either way, if the problem is low mood, avoidance, stalling, a standard nothing clears, or a voice that is reliably vicious, this is for you. Every term the book uses is explained where it first appears.

If you are in crisis, or your symptoms are severe enough that daily life is coming apart, this book is not enough on its own. Chapter 00 shows you how to tell.

THREE OPERATING RULES

1. One tool at a time, for at least a week.

Collecting techniques is the same avoidance as researching them.

2. Written down, not done in your head.

Most of these exercises lose their effect when done mentally. The writing does the work; it isn't just a record of it. If physical writing is blocked, use voice memos or digital dictation. The drill works the same.

3. Five minutes counts.

A badly done exercise beats a skipped one, permanently and by a wide margin.

Measure, don't guess. Feelings are a poor gauge of progress when mood is low, because low mood edits the memory of the week. The two-minute weekly check-in in Chapter 00 tells you whether this is working. Run it from the first week.

What this is not. This is psychoeducation and structured self-help. It is not therapy, and it is not a substitute for a clinician (a doctor, psychiatrist or therapist). The methods here come mainly from behavioural activation and cognitive behavioural therapy. Both have reasonable evidence as self-help, and both work better with some professional support than without it. [I] If you can get care at any point, this book becomes a supplement to it rather than a replacement. The bracketed marks, like [I], point to the sources in the Notes at the back.

A note on the worksheets. The book doesn't store anything you write, and that's deliberate: this is private material. Copy the worksheets in Appendix C onto paper, print them, or use whatever's actually private for you — a notes app only you open, for instance.

Before each chapter’s exercise there’s a short box marked **Stop if**. It says when to put the exercise down, and who shouldn’t do it alone. It’s the same format every time, so you can find it quickly.

That’s the operating manual for the manual. Nothing after this needs to be read in order, and nothing expires. Put it down whenever you want and open it at whichever chapter matches the day.

TRIAGE

Find Your Chapter

Start where it hurts, not at page one.

TODAY IT FEELS LIKE...	GO TO
I’m not sure I’m safe	Read This First , now
I’ve never checked whether something physical is going on	Chapter 00
The days are flat and empty	Chapter 01, then 02
I sit down and can’t start	Chapter 05, then 02
A thought like “I never finish anything” won’t let go	Chapter 03
Nothing I do is good enough to be worth doing	Chapter 04
I can’t find a reason to bother	Chapter 06
I’m going round in circles in my head	Chapter 07
Today is simply a bad day	Chapter 08
The voice in my head is vicious — my own critical thinking, not something I hear from outside myself	Chapter 09
I hear a voice, or voices, that don’t feel like my own thinking	Talk to a doctor. This book doesn’t cover it; Chapter 09 is about self-criticism, not this.
Things were better, and they’re slipping	Chapter 10
I want a structured programme	Appendix A: The Eight-Week Plan

Know Your Starting Point

Rule out what a book can't fix, set a baseline, and decide in advance when to get more help.

Every tool in this book assumes the machinery is basically sound and currently jammed. That is usually true, but not always. Ten minutes here makes sure you aren't pushing a behavioural solution against a problem that isn't behavioural.

Check the Body First

Low mood, exhaustion and difficulty starting can be driven or worsened by physical factors. If you can, ask a doctor about:

- **Thyroid function, iron and anaemia, vitamin B12 and vitamin D.** These are common, simple to test, and easy to overlook.
- **Sleep.** This includes insomnia, irregular sleep timing, and sleep apnoea (snoring, gasping, waking unrefreshed).
- **Medication side effects,** including from medicines that have nothing to do with mood.
- **Alcohol, cannabis and other substances.** These are often used to cope and often make low mood worse.
- **Hormonal changes,** including after pregnancy and around menopause, and chronic pain or illness.

If you take any medication, don't change or stop it because of anything in this book. That conversation belongs with your prescriber (the doctor or nurse who prescribed it).

Two Patterns Worth Assessing Specifically

ADHD (Attention-Deficit/Hyperactivity Disorder). Consider an assessment if difficulty starting, finishing and organising has been with you since childhood, is present even when your mood is fine, and shows up across work, home and relationships. The grading tools here help, but assessment and treatment can change what's possible in ways that practice alone can't. Adult assessment can be slow or costly to get in some places; in the meantime, the tools here still help.

Bipolar patterns. Mention it to a clinician if you've ever had stretches of several days with unusually high or irritable mood, much less need for sleep, racing thoughts, or risk-taking that was out of character for you. If this applies, keep sleep and daily routine as regular as your life allows (on shift work, as regular as your rota allows), and increase activity with professional guidance rather than on your own.

Set a Baseline: The Weekly Check-In

Same day each week, two minutes, written down. Record five things:

- **Mood:** your average for the week, 0-10.

- **Floor days:** how many days you met your minimum day, 0-7. Your minimum day is three small things that make a day count; you’ll write yours in Chapter 08. Until then, leave this blank.
- **Blocks started:** scheduled blocks started, out of scheduled blocks planned. A block is a specific time you’ve set aside for one activity (Chapter 02). Started counts; finished doesn’t matter here. Until you’ve scheduled some, leave this blank.
- **Sleep:** average hours, and whether your timing was roughly regular. On rotating shifts or nights, “regular” means regular for your own pattern, not daytime hours.
- **One line:** the most notable thing about the week.

A blank tracker is in Appendix C2. If you want a standardised measure as well, the PHQ-9 (Patient Health Questionnaire-9) is a free, widely used nine-question depression questionnaire. Many doctors use it, and it’s easy to find from reputable health services. [0] Take it at the start and every two weeks. Treat the scores as a signal for a conversation with a professional, not as a diagnosis you give yourself.

Its last question asks about thoughts of being better off dead, or of hurting yourself. If you answer anything above “not at all” on that question, go to [Read This First](#) today, whatever your total score is. That question doesn’t wait two weeks.

Decide Now: When Self-Help Isn’t Enough

Set your escalation line — the point at which you’ll get more help — while you can think clearly. On the day you need it, you’ll be inclined to argue that things aren’t bad enough yet.

IF THIS IS TRUE	DO THIS
Thoughts of death, suicide or self-harm	Read This First , today
Low mood or loss of interest most of the day, nearly every day, for two weeks or more, and it’s affecting work, relationships or basic self-care	Book an appointment with a doctor or mental health professional this week. Keep using the book alongside it.
Four to six weeks of consistent use, and the weekly check-in is flat or worse	Escalate: book that appointment with a doctor or mental health professional. This is information about the problem, not a verdict on your effort.
You can’t manage the minimum day on most days for a week	Escalate: book that appointment with a doctor or mental health professional.
You’re drinking or using more to get through the day	Escalate: book that appointment with a doctor or mental health professional, and tell them about the drinking or using specifically.

Care on a Budget

Care is sometimes more reachable than it looks. Options worth checking include your primary care doctor or GP, university or college counselling, employee assistance programmes (free counselling some employers provide, which are often unused), community and non-profit clinics, therapists who charge on a sliding scale, digital CBT programmes run by public health services, faith or community leaders you trust, and peer support groups. Appendix D covers how to use this book alongside professional help.

If cost isn't the barrier but confidentiality is — an employer who might learn about it, a residence status tied to your job — that's a real constraint, not a reason to rule out care. Ask what a service can promise about privacy before you rule it out, and look for options that sit outside your workplace, such as a community clinic or a therapist you find and pay for directly.

TRY THIS TODAY

Write your first weekly check-in now. Then write, in one sentence, the point at which you will contact a professional. That sentence is the whole assignment.

This chapter isn't exciting, and it matters. If you've written your baseline and your escalation line, the chapter's done. Stop here if you want.

PART ONE: MOVE

Behaviour is the only lever you can pull directly.

CHAPTER 01

Action Comes First

Stop waiting for the feeling. Start, and let the feeling catch up.

Action usually comes first. Motivation tends to follow.

The intuition almost everyone runs on is: feel like doing the thing, then do the thing. Under low mood, that sequence tends to reverse. Motivation stops arriving in advance and starts arriving during the task. Sometimes that's a few minutes in. Sometimes it takes days of repetition. Either way, it arrives only if you started without it.

Waiting for the feeling is therefore not patience. It's waiting for an output that, under low mood, tends to arrive only after the input.

Why the Loop Tightens

Low mood reduces activity. Reduced activity removes the sources of reward, contact and competence that would lift mood. Mood drops further, and activity drops further. The loop reinforces itself, and it doesn't need a character flaw to run. The mechanism is enough on its own.

Low mood → **Do less** → **Fewer rewards** (less contact, less lift) → **Lower mood** → *and round again*

Where the Loop Is Breakable

Not at mood, because you can't decide to feel different. Not at the thoughts either, at least not first; they tend to follow mood, and they argue back. The place to break in is **activity**, because it's the only part of the loop you can change directly by deciding to.

That is the logic of behavioural activation (BA), the method Part One is built on. In major trials for depression, delivered by therapists, it performed about as well as cognitive therapy. [1] Its core is also simple enough to run yourself, which is why it's the backbone of this book.

The practical consequence: from here on, "I don't feel like it" stops being information about whether to act. It is a weather report. It describes conditions, not instructions.

When the Lift Doesn't Come

Sometimes you do the thing and feel nothing. That isn't evidence the method failed. Reward tends to register faintly in a system that hasn't been used for a while, and in heavier depression the lift can lag by weeks. Two adjustments help:

- **Score accomplishment, not only pleasure.** A sense of having done something usually returns before enjoyment does.
- **Judge the trend over two to three weeks, not the day.** One flat afternoon tells you nothing. A flat month tells you something, and that goes to the escalation table in Chapter 00, not to a verdict about you.

Stop if starting feels physically impossible most days, not just unappealing. That belongs to the escalation table in Chapter 00, not to more effort. Stop too if this brings up thoughts of dying or hurting yourself: go to [Read This First](#).

TRY THIS TODAY

Pick one thing you've been waiting to feel like doing. Do the first two minutes of it without the feeling. Afterwards, note whether anything shifted, even slightly.

That's the mechanism the whole book runs on, and it fits in one sentence. If you read nothing else today, this chapter has already done most of its work. Close the book and let it sit.

CHAPTER 02

The Activation Protocol

Monitor, build a menu, schedule by the clock, and cut everything down until it can be started.

The protocol has three stages. Most people skip to stage three, schedule something ambitious, fail at it, and conclude the method doesn't work. Do them in order.

Stop if the log itself starts pulling your mood down. A record of empty hours can read like evidence against you. Drop to one line a day, or move on to Stage 2 with what you have. Stop too if any of this brings up thoughts of dying or hurting yourself: go to [Read This First](#). **Don't do this alone if** you noted a bipolar pattern in Chapter 00. Increase your activity gradually and with professional guidance, not by ramping this protocol up on your own, and keep sleep and routine as regular as your life allows.

Stage 1 - Monitor for Three Days

Before changing anything, record what you actually do. Each hour, write one line: what you did, and three ratings out of 10. **P** is how much pleasure it gave. **A** is how much sense of accomplishment it gave. **Mood** is your mood at that point; it's what lets you spot, afterwards, when your mood was lowest.

TIME	WHAT I DID	P	A	MOOD
09:00	Scrolled in bed	2	0	3
10:00	Made food, ate properly	4	3	4
11:00	Read the news on my phone	3	1	4
12:00	Short walk to the shop	4	2	5

Hourly is ideal. If hourly won't happen, three check-ins spread across your day are fine: for a standard day that might be late morning, afternoon and evening; on a rotating or night shift, pick three points that fit your own pattern. A rough log you keep beats a perfect one you abandon on day one. Three days is enough. Look for three things:

- Activities that score above 4 in either column.
- How much of the day goes to things scoring 0-2 in both.
- When mood is lowest, and what you were doing just before.

Stage 2 - Build the Menu

From the log, write three lists. Keep them short.

- **Reliable P:** things that scored on pleasure, however small or silly.
- **Reliable A:** things that scored on accomplishment, however trivial.
- **Low-return time:** the 0-2 activities that take up the most hours.

Add one more item to the P and A lists: *something you used to value that has quietly dropped away*. A friend you stopped messaging, a walk you stopped taking, an instrument in a cupboard. Low mood tends to prune these first, and they are often the most restorative things to bring back. If what dropped away can't come back — someone who has died, a relationship that's over — look for the part of it you could still have: the walk you used to take together, or the kind of conversation you had, with someone else.

Stage 3 - Schedule by the Clock, Not by Mood

Put items from the first two lists into specific hours, in advance. Each one is a block: a set time for one activity. This is not a to-do list. A to-do list is decided at the moment of doing, which is when you have the least capacity. A schedule is decided beforehand, and that is the entire point.

Write each item as an if-then plan: “When [time or cue], I will [small action] at [place].” For example: “When I get in from work and put my bag down, I will open one letter at the kitchen table.” Plans in this form are reliably more likely to be carried out than intentions alone. [2]

Balance the menu. Schedule at least one P item every day, not only chores. A week built entirely of obligations is a week the loop will win.

The Grading Rule (The Active Ingredient)

“Grading” here just means cutting a task down in size; nothing to do with school marks. Every scheduled item must be startable on your worst day, not your average one. “Sort out the paperwork” won't get started. “Open the top envelope” will.

TOO BIG (TRIGGERS AVOIDANCE)	GRADED DOWN TO STARTABLE
Go to the gym	Put gym clothes on
Sort out the paperwork	Open one envelope
Clean the kitchen	Clear one surface
Apply for jobs	Open the job site and read one listing
Finish the report	Open it and write one sentence
Ring the doctor about the appointment	Find the number and save it in my phone
Reply to everything	Reply to one message

TOO BIG (TRIGGERS AVOIDANCE)	GRADED DOWN TO STARTABLE
Get back in touch with people	Send one person a one-line message

An absurdly small task completed beats a reasonable one skipped. Completion is the active ingredient, not size. Once a small version is happening reliably, grade it back up one step at a time.

FIELD SHEET: YOUR ACTION MENU

RELIABLE PLEASURE (P is 4 or more)

e.g. coffee without my phone; one album start to finish

RELIABLE ACCOMPLISHMENT (A is 4 or more)

e.g. wash two dishes; send one short reply

SOMETHING THAT DROPPED AWAY

e.g. Sunday call with my brother; evening walk

LOW-RETURN DEFAULTS (0-2 on both)

e.g. refreshing feeds in bed for 40 minutes

TOMORROW'S ONE IF-THEN PLAN

When I, I will, at

IF IT'S NOT WORKING

- You keep skipping the log. Drop to three check-ins a day. Partial data is still data.
- Things are scheduled but not started. They're graded wrong. Go smaller, and see Chapter 05.
- Things get done but there's no lift. Read "When the Lift Doesn't Come" in Chapter 01. Track the A column, and judge the trend over weeks.
- The schedule is all obligations. Add one P item a day, non-negotiable.

This is a long chapter, and running Stage 1 is a complete session on its own. You don't need the rest today. Stop here if stopping is what's available.

PART TWO: THINK

Treat the loudest thoughts as claims, and check them.

CHAPTER 03

Questioning the Failure Thought

Not positive thinking. Accurate thinking, checked on paper.

Some thoughts arrive fully formed and feel like observations rather than opinions. They never get examined, because they don't announce themselves as claims. "I've wasted years." "Everyone my age is ahead." "I never finish anything." They present as facts about reality. They are actually predictions and evaluations, and both can be tested.

The method is not positive thinking, which rarely works and which you'd probably reject anyway. It is treating the thought as a claim and checking it against evidence, the way you'd check any other claim. [3]

Common Shapes

Distorted thoughts tend to follow a small number of patterns. Naming the pattern is often half the work.

SHAPE	SOUNDS LIKE
All-or-nothing	"If it isn't excellent, it's worthless."
Overgeneralising	"I never finish anything."
Mind reading	"They can tell there's something wrong with me."
Fortune telling	"This will fail like the last one."
Discounting	"That one worked out, but it doesn't count."
Labelling	"I'm a failure," instead of "this attempt failed."

The Thought Record (Written, Not Mental)

COLUMN	WHAT GOES IN IT
Situation	What was happening, factually, in one line
Emotion	What you felt, named in one word, rated 0-100
Thought	The exact sentence, in the words it arrived in
Belief	How true it feels right now, 0-100%
Evidence for	Facts that genuinely support it. Be fair here.

COLUMN	WHAT GOES IN IT
Evidence against	Facts that don't fit it, including ones you'd normally discount

The thought record rates out of 100, not out of 10 like the rest of the book, because small shifts matter here.

Then write one sentence that accounts for both evidence columns, and re-rate both the belief and the emotion. The emotion rating is the one that actually matters. The belief is how you get to it.

WORKED EXAMPLE ONE — THE AUTHOR'S OWN

- **Situation:** Sat down to work at 09:45. Still sitting there at 11:00, having done nothing.
- **Emotion:** Shame, 80
- **Thought:** "I never finish anything. That's just what I am."
- **Belief:** 90%
- **Evidence for:** Two products abandoned or stalled. No public launch to date. Several restarts from zero.
- **Evidence against:** One app has a dozen features built and deployed. A second product is live, with a brand document and positioning. A university degree, finished. Both stalls had identifiable practical causes, not a loss of ability. The pattern is stalling at a specific point, not a general inability to finish.
- **Balanced sentence:** "I stall at the point where a project needs money or an outside party, and I haven't yet pushed through that point. That's a specific, fixable failure mode, not a description of my character."
- **Re-rate:** Belief 65%. Shame 55.

WORKED EXAMPLE TWO — SIX YEARS IN THE SAME JOB

- **Situation:** Six years in the same role. A colleague two years behind me was promoted past me last month.
- **Emotion:** Regret, 70
- **Thought:** "I've wasted years here. Everyone my age is ahead of me."
- **Belief:** 80%
- **Evidence for:** Six years without a promotion. One colleague promoted past me. No move of my own in that time.
- **Evidence against:** I changed teams twice and picked up skills neither of my past roles used. My manager has twice said, outside any review, that my work is the strongest on the team. "Everyone my age" turns out to be a handful of people I compare myself to, not everyone; two of them have told me they'd swap for my job. Comparing years served to one person's promotion ignores what the years were actually for.

- **Balanced sentence:** “I haven’t been promoted in six years, and I’ve also built skills nobody’s asked me to use yet. Both are true, and neither one makes the years ‘wasted.’”
- **Re-rate:** Belief 65%. Regret 55.

WORKED EXAMPLE THREE — A SHARED HOME

- **Situation:** Saturday morning. Two weeks of dishes, unopened post and laundry. My partner’s mother is visiting tomorrow.
- **Emotion:** Shame, 85
- **Thought:** “Everyone can see I’m failing at the basics. They must think I’m useless.”
- **Belief:** 90%
- **Evidence for:** The house is a mess. Bills are unopened. My partner sighed at the sink last night.
- **Evidence against:** Nobody has said “useless”. The sigh could mean tired, and I haven’t asked. Three weeks ago the house was fine, and it has been fine for most of the years we’ve lived here. My partner has let things slide this fortnight too. The sink takes twelve minutes, not three weeks.
- **Balanced sentence:** “The house has slid for two weeks, and I’m guessing at what people think of me (mind reading). I can clear the sink today, and ask about the sigh instead of guessing.”
- **Re-rate:** Belief 65%. Shame 60.

WORKED EXAMPLE FOUR — MONEY THAT’S ACTUALLY SHORT

- **Situation:** Three overdue notices unread on the desk for a month. I already know the account can’t cover all of them.
- **Emotion:** Dread, 80
- **Thought:** “I’ve ruined my finances permanently and there’s no way out.”
- **Belief:** 85%
- **Evidence for:** The money is short. The deadlines have passed and late fees will apply. I ignored two reminders.
- **Evidence against:** “Permanently” is a prediction, not a fact. Two of the letters are automated reminders, not final demands. I haven’t asked any of the three about paying in instalments, so I don’t yet know what’s possible.
- **Balanced sentence:** “I’m genuinely behind, and it’s going to cost me. I don’t yet know it’s permanent, because I haven’t opened the letters or asked what my options are.”
- **Re-rate:** Belief 75%. Dread 70.

The shape is the same across all four examples. The situations differ, but the mechanism doesn’t: an over-general claim, once tested, comes back narrower and more accurate. It never comes back comfortable, just more correct.

Calibrate your expectations. A drop of 10 to 30 points on the emotion rating is a good result; that's the rating that matters most (see above). You aren't trying to reach zero. You're trying to get the thought down to the size the evidence supports. Some records will barely move, like Example Four, especially early on or on the worst days. Those still count, and the effect builds with repetition.

When not to use this. If the thought is accurate ("I'm two months behind on rent"), it doesn't need challenging. It needs a decision: what's the one next step you can take today, however small? If it's money, that's often opening the letter and asking about paying in instalments; many countries also have free debt-advice services. Testing true thoughts only produces frustration. Example Four shows the edge of this: once most of the thought turned out to be true, the useful move was a decision, not more evidence. If the thought is about wanting to die, or about hurting yourself, this tool isn't the right one either: go to [Read This First](#) instead of writing it down alone.

Stop if writing the thought down makes it louder, your distress spikes sharply, or the thought is about dying or hurting yourself. Put the record down and go to [Read This First](#). If you're not sure whether a thought needs this record or a person, use a person.

FIELD DRILL: THOUGHT RECORD

1. SITUATION (FACTS ONLY)

Who, what, where, when

2. EMOTION AND INTENSITY (0-100)

e.g. shame, 80

3. THE THOUGHT, WORD FOR WORD

The exact sentence in your head

4. BELIEF (0-100%)

e.g. 90

5. EVIDENCE FOR (FACTS ONLY)

Facts that support the thought

6. EVIDENCE AGAINST (INCLUDING WHAT YOU'D NORMALLY DISCOUNT)

Facts that don't fit

7. BALANCED SENTENCE

One sentence that accounts for both columns

8. RE-RATE BELIEF AND EMOTION

e.g. belief 65%, shame 55

That's the tool. The first few take longer than ten minutes, and they're genuinely tiring. That's normal, and not a sign you're doing it wrong. A good place to stop.

CHAPTER 04

Calibrating the Standard

High standards improve work. Impossible ones prevent it. Here's how to tell them apart and test them.

High standards improve work. Impossible standards prevent it. They are easy to confuse because they use the same vocabulary, and because the second one feels more virtuous than the first. Clinically, the problem isn't having high standards. It's when your sense of self-worth depends on meeting them, and you keep them in place despite the obvious costs. [4]

The Test (Four Questions)

1. **Has anything you've done ever cleared it?** If nothing ever has, it isn't calibrated to reality.
2. **Would you apply it to someone else's first attempt?** If not, it isn't a standard. It's a rule with one target.
3. **Does it tell you what to do, or only what's insufficient?** Usable standards say what to do. Impossible ones only veto.
4. **When you meet it, does it stay met?** Or does the bar move up so the verdict stays the same?

A standard that fails questions 1 and 4 is not measuring your work. It is producing a verdict decided in advance, and the work is a formality.

Where These Can Come From

For some people, standards like this were shaped early, calibrated against approval that was scarce or depended on performance. If that fits you, it may explain why meeting the bar produces relief rather than satisfaction, and why the relief doesn't last. If it doesn't fit, that's fine too. You don't need an origin story for any of this to work. The tools target what the standard does now, not where it came from, and arguing with its history rarely loosens it.

Scope It Instead

Don't attack the standard. Demote it from a veto on each individual thing to a rule for your work as a whole.

APPLIED AS	REAL-WORLD EFFECT
Every single thing must clear it, before you start	Nothing starts. The standard becomes a gate at the entrance.
Your work as a whole should clear it, over years	Things get done. Some are practice. The standard becomes a direction.

The second framing doesn't lower your ambition. It's the only version in which ambition can come out as work rather than paralysis.

Run One Experiment

Arguing with a standard changes little. Testing it can. [4] Deliberately let one thing go at 80%: a message sent without the third reread, a meal that's good rather than perfect, a room tidied rather than spotless, a report handed in without the final polish.

Stop if the standard covers safety-critical work — medication doses, sign-offs, legal filings, anything where getting it wrong could hurt someone. Don't run the 80% experiment there. Stop too if this brings up thoughts of dying or hurting yourself: go to [Read This First](#). **Don't do this alone if** your checking has the repetitive, can't-stop quality of OCD (obsessive-compulsive disorder) rather than ordinary high standards. Deliberately checking less is a real treatment for that, but it's one to do with a therapist.

1. **Before:** write down what you predict will happen, and how sure you are, 0-100%.
2. **Do it.** Don't fix it afterwards.
3. **After:** write down what actually happened. Compare it with the prediction.

Most people find the feared consequence either didn't happen or was much smaller than predicted. Each result is a data point the standard has to account for, and it can't absorb these the way it absorbs arguments.

Sometimes the feared thing does happen. Write that down too: what it actually cost, and what you did about it. One bad result is one data point. It doesn't hand the standard its veto back.

The practical move: pick one thing you're working on — at work, at home, anywhere. Decide once, in writing, whether it's a practice run or one that has to clear the bar. Then hold that decision. If it's a practice run, the bar doesn't get to vote on it again. Re-opening the question mid-task is the thing to notice and stop, not the thing to settle.

TRY THIS TODAY

Choose one small 80% experiment. Write down your prediction. Run it within 24 hours.

This chapter can be hard to accept, because the standard will tell you that accepting it is a compromise. Let it sit for a day before moving on. None of this needs settling tonight.

PART THREE: START

The instant between scheduled and begun.

CHAPTER 05

The Not-Starting Moment

The resistance is attached to starting, not to the task. Get past the start.

There is a specific instant when the task is scheduled, you are physically present, and you don't begin. Everything else in this manual is preparation for that instant.

What it actually feels like. Not laziness; laziness is comfortable, and this isn't. It's rising pressure in the chest or gut, a sudden conviction that you need to check something first, a strong pull toward any other screen. It is aversion, and it behaves like other urges: it rises, peaks and usually fades within minutes, whether or not you act on it.

Research on procrastination describes it as short-term mood repair. Avoiding the task makes the bad feeling drop immediately, and that immediate relief is exactly what gets reinforced. [5] Each escape teaches your nervous system that the task really was a threat.

Urge Surfing, Applied to Avoidance

Urge surfing comes from work on cravings, such as for cigarettes or alcohol, where it's used to ride out an urge without acting on it. [5] Here it's adapted: instead of escaping the feeling, you let it run without obeying it.

Check your body first. New or unusual chest pressure — especially with shortness of breath, pain spreading to your arm or jaw, or a racing heart that doesn't fit the moment — needs a doctor before it needs this chapter (see Chapter 00, Check the Body First).

Stop if ninety seconds of watching makes the feeling climb rather than ease, or tips into panic. That isn't doing it wrong; some bodies do this. Stop watching and ground yourself instead: feet on the floor, cold water on your hands, name five things you can see. Stop too if this brings up thoughts of dying or hurting yourself: go to [Read This First](#). **Don't do this alone if** you have a history of panic attacks or trauma.

1. **Name it:** "This is avoidance, not a decision."
2. **Locate it in your body:** chest, throat or stomach. Find where it actually sits.
3. **Don't act for ninety seconds.** Don't argue with it, don't start the task, don't escape. Just watch it.
4. **Notice it shift.** It usually peaks and starts to ease. That is the thing you're proving: the feeling and the behaviour can be separated.
5. **Then take the smallest possible step toward the task.** Surfing is the on-ramp, not the destination. Watching the urge indefinitely is just a more mindful way of not starting.

The Five-Minute Rule

Commit to five minutes of the task, with explicit, genuine permission to stop at the end. Set a timer.

This is not a trick to get you to work for an hour. If it becomes one, it stops working, because you'll learn the permission was fake. Stop at five minutes when you want to. That's what keeps the mechanism honest.

Why it works: the cost of starting is almost entirely at the front. The aversion is attached to the transition, not the activity. A couple of minutes in, the thing generating the resistance has usually gone.

If five minutes is too much, the task is graded wrong. Cut it down until the first step is something like opening the file, finding the form, picking up the first thing, or standing up. No size is too small to be worth doing. A badly graded task you avoid teaches avoidance, and a trivial one you complete teaches completion. Only one of those builds in your favour.

Pre-Decide the Hard Moment

Write one if-then plan for the exact moment you tend to escape: "If I notice myself reaching for my phone at the start of a block, then I put it out of reach — another room, a bag, a drawer — and start the five-minute timer." A decision made in advance doesn't have to be made under pressure.

TRY THIS TODAY

At your next scheduled block: name it, ninety seconds, smallest step, five-minute timer. Write down what happened.

You'll probably come back to this chapter more than any other, and you now have enough to get through tomorrow morning. Close the book here with a clear conscience. Everything after this builds on it.

CHAPTER 06

Values Over Goals

A goal is a destination. A value is a direction you can move in today.

Goals are achieved and then gone. Values are directions you can move in today, whatever the outcome. [6] The distinction sounds soft, but it does real work: on a day when no goal feels reachable, a value still tells you what to do.

DIMENSION	GOAL (DESTINATION)	VALUE (DIRECTION)
Example	Get the house sorted by Sunday	Live somewhere that isn't working against me
Example	Find a job by March	Use what I'm good at, wherever I am

DIMENSION	GOAL (DESTINATION)	VALUE (DIRECTION)
Status	Achieved or failed	Ongoing; can't be failed
On a bad day	Feels impossibly far away	Still actionable in five minutes
When reached	Gone, and needs replacing	Unchanged; keeps directing

Why this matters. When self-worth is loaded onto finishing goals, every unfinished thing becomes evidence about you as a person. That makes starting expensive, because starting risks a verdict. Acting from a value takes the verdict out of the transaction. You aren't proving anything today; you're pointed in a direction today.

Finding Yours: Two Questions

First: think of a time you were absorbed in something and lost track of time. Not a proud memory; an absorbed one. What was the activity actually doing? Solving a puzzle? Making something exist that didn't before? Looking after someone? Being with people you like? Understanding how something works, all the way down?

Second: what irritates you in other people's work, or in how people treat each other? Irritation at sloppiness points to a value of craft. Irritation at things that shut people out points to a value of access: people being let in. Irritation at unkindness points to a value of care. Your annoyances are often a more honest signal than your aspirations.

Write three values, no more, in your own words. The should-check: if a value sounds like something you'd say to impress someone, or something you think you ought to care about, it's probably a "should", not a value. Cross it out. If everything gets crossed out, that's common when mood is low. Keep one anyway and test it for a week; values get clearer by acting on them, not by thinking about them.

Stop if looking for what you value brings up mostly loss — people, work or a life that's gone. That's grief, not a failed exercise, and it's worth talking to someone about. Stop too if it brings up thoughts of dying or hurting yourself: go to [Read This First](#).

FIELD SHEET: THREE DIRECTIONS

VALUE 1, AND ONE FIVE-MINUTE ACTION THAT SERVES IT

e.g. Craft — fix one thing properly, not quickly. Care — ring someone and actually listen.

VALUE 2, AND ONE FIVE-MINUTE ACTION

VALUE 3, AND ONE FIVE-MINUTE ACTION

Using them. When a block is scheduled and you can’t find a reason to begin, don’t reach for the goal. It’s too far away, and it carries a verdict. Reach for the value instead: “These next five minutes are me doing this properly.” That sentence becomes true the moment you start, which is what makes it usable when nothing else is.

Three values, written down, and this chapter has done its job. There’s no benefit to continuing today.

PART FOUR: HOLD

Staying on the road when the head, the day, or the inner voice turns against you.

CHAPTER 07

The Rumination Trap

If a decision or action wouldn’t end it, it isn’t thinking. It’s circling.

Rumination — going round in circles in your head — is the most convincing form of avoidance there is, because it looks exactly like thinking hard about the problem. It feels productive, and it reliably produces nothing. Research on repetitive thinking suggests the key difference isn’t how much you think but how: abstract “why” questions keep the loop going, while concrete “what” and “how” questions tend to end it. [7]

The Test That Separates Them

One question: would a decision or an action end this?

PROBLEM-SOLVING	RUMINATION
About something that can change	About something already fixed, or abstract
“What do I do about X?”	“Why am I like this?”
Ends in an action	Ends when you get tired
Feels like narrowing down	Feels like circling
Ten minutes usually does it	Forty minutes, and you’re further back

If the same thought has come round twice with no new information, it's rumination. A third pass confirms it.

Stop if what's circling is a thought about dying or hurting yourself, or a memory you're re-living rather than reviewing. None of the interrupts below is the tool for that: go to [Read This First](#), or to a doctor or therapist. **Don't do this alone if** the loops are about something traumatic that happened to you, or they have the stuck quality of obsessions: thoughts you don't want and can't dismiss.

Three Interrupts (Strongest First)

1. **Go concrete.** Rewrite the loop as a specific question with a possible answer. "Why am I always behind?" doesn't convert. "What exactly happened this week, and what's the one thing blocking Monday?" does. Answer it in writing, pick the next step, and stop. If a loop won't convert at all, it isn't a problem, it's a mood, and moods respond to activity, not analysis. Go to Chapter 02. This doesn't apply to a loop about wanting to die or to hurt yourself; that isn't a mood to wait out. Go to [Read This First](#) instead.
2. **Name it and move.** Say "circling", then change your body. Stand up, walk to another room, go outside, put something on. If you can't move far, change something where you are: wash your face, run cold water over your hands, sit differently. The state is partly physical, and changing posture interrupts it more reliably than arguing with what it's saying.
3. **Postpone it.** Set fifteen minutes at the same point in your day, every day; on a rotating or shift pattern, pick a fixed point in your own cycle rather than a clock time. When the loop starts outside that window, write the topic on a list and stop. At the appointed time, sit down and think about the list on purpose. Many items have lost their pull by then, which is a lesson in itself. This technique was developed for anxious worry, and it's a useful backup for depressive loops. [7] Don't put thoughts of dying, hurting yourself, or a trauma memory on this list to think about later. Those go to [Read This First](#) or a professional now, not onto a schedule.

The Trap for People Who Read About Psychology

Understanding your own mechanisms is genuinely useful. It is also the best fuel rumination can get. Analysing a pattern can stand in for changing it indefinitely, and it feels like progress the whole time. You don't need to have read any psychology for this to apply: any thinking that stands in for doing works the same way.

The check: has this analysis changed anything I did this week? If not, it's the loop wearing a lab coat.

Notice if you're now inclined to read straight on rather than try any of this. That's worth noticing, not judging; the chapter's point stands either way. If you can, put the book down and try one interrupt before you turn the page.

CHAPTER 08

The Bad-Day Protocol

Define the floor in advance. Come back before a gap reaches two days.

Decide this now, while you're able to decide things. On the day you need it, you won't be able to design it.

Write your minimum day: three items. They aren't work items. They're the floor: hit them, and the day counts. Something like: get up and dressed, eat one real meal, go outside once. That's the whole bar. Hitting three tiny things on a bad day is a completed day, not a failed one.

The Rule That Matters: Never Miss Twice

A missed day means a day you didn't hit your minimum day at all — not a day you did a smaller version, and not a day covered by “a genuinely bad stretch” below.

One missed day is an event. Two in a row make a third easier to miss, and the longer a gap runs the more it costs to restart. So the target isn't perfection. It's making sure a gap doesn't run past one day if you can help it.

One well-known study found that missing a single opportunity didn't meaningfully set back how habits formed. [8] That study is about one miss, not about what happens after two or three; this rule is the book's own judgement about momentum, not something the study measured directly. What matters, either way, is coming back, not keeping an unbroken streak.

After one missed day, there are two roads:

- **Resume at the minimum** → the gap closes.
- **Miss again on day 2** → it gets harder to restart.

Resuming Without the Restart Ritual

After a gap, the temptation is to relaunch: a new plan, a clean slate, a bigger commitment to make up for lost time. That's the collapse-and-restart cycle, and the oversized recommitment is what guarantees the next collapse.

Instead, resume at the minimum. Do one block, graded down small, on the next scheduled day. Nothing is rebuilt, nothing is made up for, and the plan isn't revised. You just rejoin it.

What a Gap Is Actually Telling You

THE GAP CAME FROM	THE FIX
Blocks too big to start	Grade them down further
Blocks that don't connect to anything you care about	Check them against your values (Chapter 06)
A genuinely bad stretch: low mood, illness, a hard week	Nothing structural. Run the minimum day and wait it out; this is real permission, not a loophole. If the stretch doesn't lift, use the escalation line in Chapter 00 alongside it. Waiting it out and escalating aren't opposites.

People most often get the third case wrong. Sometimes the answer is that the week was bad and no change to the system is needed. Redesigning the system after every bad week is how you end up with a system you've never actually run.

Stop if the minimum day itself is out of reach on most days for a week. That's past what this chapter covers: use the escalation table in Chapter 00. Stop too if a bad day brings thoughts of dying or hurting yourself: go to [Read This First](#).

FIELD SHEET: MY MINIMUM DAY

1. BODY CARE

e.g. wash face, brush teeth, change clothes

2. FUEL

e.g. one real meal, a glass of water

3. OUTSIDE OR CONTACT

e.g. two minutes of daylight, or one message to a person

Write your three minimum items now, while you're able to, and then stop. That's a complete session, and there's nothing else this chapter needs from you.

CHAPTER 09

Self-Attack as a Defence

Self-loathing often isn't a conclusion. It's a move, and moves can be changed.

This chapter is about the critical voice in your own head: the one that argues, judges and won't let up. If you hear a voice, or voices, that don't feel like your own thinking, that's a different thing, and worth mentioning to a doctor rather than working through here.

If the self-attack has moved from words to actions, such as hurting yourself, or you're thinking about it, this chapter isn't the tool right now. Go to [Read This First](#); it covers this, not only suicidal crisis. Come back to this chapter once you're safe.

Self-loathing is not usually a symptom sitting there doing nothing. Often it's a behaviour, kept going because it works. Looking at what it does for you (its function) is more useful than debating what it says. None of this requires you to be weak or self-indulgent. It only requires the mechanism to work, which it does.

What it buys you. Three things, all real, all delivered immediately:

- **Getting in first.** If you deliver the verdict first, nobody else's can land as hard. You can't be caught out by a judgement you've already made.
- **Avoiding the test.** As long as the question is settled, you never have to run the experiment that might answer it badly. Conceding the point protects you from finding out.
- **The feel of integrity.** Self-criticism tends to be read, by you and by others, as honesty, humility and accountability. It's a form of avoidance that feels like a virtue from the inside. Real humility is different: it's accurate about you, the good included, and it leaves you able to act. Self-attack borrows its reputation.

That's a strong reward: immediate relief, a delayed cost, and the sense that it's the right thing to do. Anything rewarded like that gets repeated often.

Why arguing with it fails. You can't refute it, because it isn't a conclusion. It's a move. Counter-evidence gets absorbed ("that doesn't count"), because its job is protection, not accuracy. That's why arguing with it, or reading about it, hasn't dented it. You've been debating something that was never making an argument.

Stop if this brings up thoughts of dying or hurting yourself, or a memory rather than a thought. Put it down and go to [Read This First](#), or to a doctor or therapist. That isn't failing at this chapter; it's using it correctly. **Don't do this alone** if the voice echoes, closely, someone who was cruel, controlling or abusive to you. Looking straight at it is heavier when it has a real source. Do it with a therapist if you can, and not for the first time on a hard day.

The Tool: Cost-Benefit, Honestly Filled

Do this for the behaviour, not for yourself. Fill in both columns properly; a rigged one teaches nothing. The table below is a worked example, to show the shape of it, not a verdict on you. Fill in your own version in Appendix C5, in your own words.

BENEFITS OF SELF-ATTACK	COSTS OF SELF-ATTACK
Nobody can judge me worse than I already have	Nothing I do can ever count; the verdict is pre-written
I never risk trying and being found wanting	Avoiding the test means the question stays open forever
It feels rigorous and honest	It can't be proven wrong, which is the opposite of rigour
I'm braced for everyone else's judgement	I meet people already braced, and it gets between us
It's familiar; I know how to do this	It keeps costing time the protection was supposed to save — not a verdict on the years already gone, just a reason to stop paying now

Read the right-hand column as a price list, because that's what it is. The protection isn't free, and it isn't cheap.

Three Signs It's Defence, Not Assessment

1. **It's global.** "Incompetent in every way." Real assessment is specific and names a domain.
2. **It arrives before the evidence.** Verdict first, then a search for support.
3. **It never updates.** Nothing you could do would change it, which means it isn't tracking you at all.

Changing the Response in the Moment

This isn't positive self-talk. You'd reject that, correctly, because it's false and obviously a technique. Four steps instead:

1. **Label the move, not the content.** Say "That's the defence," not "that's not true." You're naming what it does, and that's much harder for it to absorb.
2. **Delay; don't debate.** Give it ten minutes before engaging. Urges fade, and most of the time the conversation never resumes.
3. **Replace it with something specific and true.** Not "I'm capable." Something narrow enough to be accurate: "I don't know yet whether I can finish this. I haven't run the test." Uncertainty is honest, and it leaves the question open, which is exactly what the defence exists to prevent.
4. **Change the tone, not only the words.** Say the accurate sentence the way a decent coach would say it to someone they're backing: just as honest, without the contempt. People tend to skip this step, and it matters. In compassion-focused approaches, the tone of the inner voice is a target in its own right, because the same accurate sentence lands differently when it isn't delivered as an attack. [9]

If warmth feels impossible, try distance instead. Write the sentence about yourself in the third person, using your own name, the way you'd write about a friend: "Sara doesn't know yet whether she can finish this", not "I don't know". Talking to yourself this way tends to make hard feedback easier to hear without flinching. [9]

The point isn't to feel good about yourself. It's to stop closing the question in advance, so that evidence can finally start to land on it. You don't have to win the argument. You have to stop having it long enough to make something, or to walk into a room without the verdict already delivered.

This chapter goes deep, and it doesn't need to land in one reading. If it's hit hard, that's worth telling someone — a friend, or a therapist; compassion-focused therapy (Appendix D) is built for exactly this. Put it down for now if you need to. You'll be better placed to use it later, and none of this has to be settled tonight.

CHAPTER 10

Keeping It

Getting better and staying better are different skills. This is the second one.

Improvement is rarely a straight line. Low mood tends to come back through the same doors it left by, and usually quietly. The goal of this chapter is to spot it at the door rather than in the living room. [10]

1. Your Early Warning Signs

What slips first for you? Common ones:

- Sleep timing drifts later (on shifts: drifts away from your usual pattern), or sleep gets longer and less restful.
- You stop replying to people, starting with the easy ones.
- The log or the weekly check-in quietly stops.
- Rumination time creeps up, often disguised as "planning".
- The restart fantasy appears: a new system, starting Monday.

Look back at your own history and write down your top three. Be specific. "Low mood" is too late to be a warning sign. "Skipping my Sunday call" is early enough to act on.

2. Your First Moves

Decide in advance what you'll do when two warning signs appear in the same week. A good default:

1. Go back to three days of monitoring (Chapter 02).
2. Make the minimum day non-negotiable (Chapter 08).
3. Make one real contact with one person, within 48 hours.

3. Your Escalation Line

Copy it from Chapter 00 and put it somewhere you'll see it. Name the specific person or service you'll contact, and how. If you haven't filled in the Safety Plan (Appendix C4), do it alongside this; the two belong together.

4. Maintenance

On good weeks, keep one tool running: the weekly check-in. It costs two minutes, and it's the instrument that catches the drift before you feel it.

FIELD SHEET: MY PERSONAL PLAN

MY THREE EARLY WARNING SIGNS

- 1.
- 2.
- 3.

MY FIRST MOVES WHEN TWO APPEAR IN ONE WEEK

What I'll do, in order:

MY ESCALATION LINE, AND WHO I'LL CONTACT

If, I will contact by

A plan written on a good day is a gift to yourself on a bad one. Write it, put it somewhere visible, and close the book.

APPENDIX A

The Eight-Week Plan

For readers who want structure. One new thing per week, with everything earlier kept running.

WEEK	READ	DO
1	Chapters 00, 01; Ch. 02 Stage 1	Baseline check-in. Write your escalation line. Fill in the Safety Plan (Appendix C4) while you're at it; it's worth having before you need it. Monitor for three days.
2	Ch. 02 Stages 2-3; Ch. 08	Build the menu. Schedule one graded block a day as an if-then plan. Write your minimum day.

WEEK	READ	DO
3	Ch. 05	Use the not-starting routine at every block. If Week 2 held, go to two blocks a day. If you noted a bipolar pattern in Chapter 00, increase activity with professional guidance rather than by this schedule alone.
4	Ch. 03	Three written thought records this week.
5	Chapters 04, 06	Write your three values. Decide whether what you're working on is a practice run or has to clear the bar. Run one 80% experiment.
6	Ch. 07	Practise the three interrupts whenever you catch yourself circling.
7	Ch. 09	Fill in one cost-benefit (Appendix C5). Use the four steps when the voice turns up.
8	Ch. 10	Write your personal plan. Compare your Week 1 and Week 8 check-ins, then decide what continues.

Repeating a week isn't falling behind. It's part of the protocol. If a week didn't hold, run it again at a smaller size before adding anything new. If the check-in is flat or worse by weeks 4-6, use the escalation table in Chapter 00. That isn't a failure of the plan; it's the plan working as designed.

APPENDIX B

Quick Reference

In crisis right now? US and Canada: 911, or call or text 988. UK: 999. UK and Ireland: Samaritans on 116 123. EU: 112. Australia: 000, or Lifeline on 13 11 14 (text 0477 13 11 14). UAE: 999 for police, 998 for an ambulance. Anywhere else: findahelpline.com. More in [Read This First](#).

Thirty seconds. Find the row that matches, and do what the right-hand column says.

WHEN THIS HAPPENS	DO THIS IMMEDIATELY
Thoughts of harming yourself	Read This First . Contact a person or service now; the numbers are above.

WHEN THIS HAPPENS	DO THIS IMMEDIATELY
“I don’t feel like it”	Start anyway. The feeling usually arrives during, not before.
Did it, felt nothing	Score accomplishment, not pleasure. Judge over weeks, not days.
Sitting there, not starting	Name it. Ninety seconds without acting. Smallest step. Five-minute timer, with permission to stop.
Keep failing the same block	It’s too big. Cut it down until the first step is tiny: open the file, pick up one thing.
“I never finish anything”	Write a thought record: emotion, evidence for and against, one balanced sentence, re-rate.
“This isn’t good enough to be worth doing”	Practice run, or one that has to clear the bar? You already decided. Don’t reopen it now.
Circling the same thought	Go concrete: “What exactly, and what’s the next step?” If it won’t convert, stand up and move.
Can’t find a reason to start	Skip the goal and use the value: “These five minutes are me doing this properly.”
The inner voice is vicious	Label it as the defence. Wait ten minutes. Say something specific and true, in a coach’s tone.
Bad day	Run the minimum day. Three small things counts as a complete day.
Missed yesterday	Resume today at the minimum. No relaunch. Never miss twice.
Analysed it thoroughly, changed nothing	That’s the loop in a lab coat. Pick one action and do it badly.
Two warning signs in one week	Run your first moves from Chapter 10.

THE THREE THAT CARRY MOST OF THE WEIGHT

1. Action usually comes before motivation. Don’t wait for the feeling.
2. Never miss twice.
3. A task cut down until it can be started beats a correctly sized task that gets skipped.

APPENDIX C

Worksheets

Print these, or copy them into a notebook or a notes app only you open. The book doesn't store anything you write.

C1 - Activity Log (one day)

Write the time in the first column: hourly if you can, or three check-ins that fit your day.

TIME	WHAT I DID	P (0-10)	A (0-10)	MOOD (0-10)

C2 - Weekly Check-In

WEEK	MOOD (0-10)	FLOOR DAYS (0-7)	BLOCKS STARTED / PLANNED	SLEEP (HRS, REGULAR?)	ONE LINE
1					
2					
3					
4					

WEEK	MOOD (0-10)	FLOOR DAYS (0-7)	BLOCKS STARTED / PLANNED	SLEEP (HRS, REGULAR?)	ONE LINE
5					
6					
7					
8					

C3 - Thought Record

COLUMN	ENTRY
Situation	
Emotion (0-100)	
Thought (word for word)	
Belief (0-100%)	
Evidence for	
Evidence against	
Balanced sentence	
Re-rate belief / emotion	

C4 - Safety Plan

Fill this in on a steadier day, ideally with someone you trust or a clinician. Work through the steps in order: if one doesn't lower the danger, go to the next rather than stopping. Keep it where you can reach it fast and where it stays private: on paper, or in a notes app only you open. If you share a home or a phone, choose where it lives, and who else holds a copy, with that in mind. The structure is adapted from a widely used, evidence-based safety planning approach. [C]

STEP	MY ENTRIES
1. My warning signs Thoughts, moods, situations that tell me a crisis may be building	

STEP	MY ENTRIES
2. Things I can do on my own To take my mind off things or ride out the feeling	
3. People and places that help me feel less alone Names, or where I'd go	
4. People I can ask for help Names and numbers	
5. Professionals and crisis lines My clinician, local emergency number, crisis line. Starting points: Read This First and the box at the top of Appendix B.	
6. Making my surroundings safer What I will put out of reach or ask someone to hold, including stored medication or anything else specific to how I might hurt myself. Also: not being alone, and going easy on alcohol, for now.	
7. One reason worth staying for It's fine to leave this blank for now, or to fill it in with someone else.	

C5 - Cost-Benefit of Self-Attack

From Chapter 09. Fill in both columns honestly, in your own words.

BENEFITS OF SELF-ATTACK	COSTS OF SELF-ATTACK

APPENDIX D

Working With a Professional

This book works better alongside help than instead of it.

Bring the Paperwork

Your activity logs, weekly check-ins and thought records are exactly what a clinician would otherwise spend sessions trying to reconstruct. Bring them. They shorten the assessment and make the first sessions far more useful.

Approaches Worth Asking About

- **Behavioural activation (BA)** - Part One of this book, delivered with guidance.
- **Cognitive behavioural therapy (CBT)** - Parts One and Two, plus much more.
- **Acceptance and commitment therapy (ACT)** - the values work and urge surfing, in depth.
- **Compassion-focused therapy (CFT)** - particularly relevant if Chapter 09 hit hard.
- **Interpersonal therapy (IPT)** - if low mood is tied to relationships, loss or a major change in role.
- **Medication** - a conversation with a prescriber, and often combined with therapy for moderate to severe depression.

Questions to Ask

- “What approach do you use, and what’s the evidence for it with what I’m dealing with?”
- “How will we know if it’s working, and when would we change course?”
- “What should I be doing between sessions?”

A good clinician will welcome these questions. If one doesn’t, say so, or ask for a second opinion, rather than dropping out of care altogether.

NOTES

Sources and Further Reading

The bracketed numbers and letters in the text, like [1], [0] or [C], point to the sources below, organised by chapter.

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CHAPTER 10 [10]

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APPENDIX C [C]

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CLOSING NOTE

Starting from Jammed

The standing reminder: this manual is scaffolding, not treatment. If care becomes reachable, whether a free service, a subsidised one or a clinician later on, take it, and let this become a supplement rather than a substitute.

And the last thing. Nothing in this book asks you to become a different person. Every tool in it assumes the machinery is sound and currently jammed, because that is usually what's true.

You don't have to believe that to use them. Procedures can work before you're convinced by them, which is exactly what makes them useful on the days you're convinced of nothing.

You are not starting from zero. You are starting from jammed, and there is a way out of jammed.

Pick one tool. Do it badly. That's the whole instruction.

Tools, Not Theory — Mohammad Haris

Fieldwork Press — First Edition 2026